

Q4.1a Case Study 1 - Improving Diabetes Care for an Asian-Heritage Cohort

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Why this group is a priority (local need)

Over half of Sudbury's registered patients are of Asian heritage and Brent diabetes prevalence (8.58%) exceeds England (7.26%), with earlier onset and higher complication risk in South Asian communities. Closing this gap is a Core20PLUS5 priority.

Potential barriers to access

- Language and health-literacy barriers (around one in three not using English as a main language).
- Cultural factors in diet, fasting and self-management.
- Lower uptake of structured reviews and retinal/foot screening in some groups.

Specific measures to address barriers

- Bilingual recall and education materials; interpreter-supported reviews.
- Culturally tailored self-management and dietary advice (including Ramadan guidance) with community partners.
- Pharmacist-led structured medication reviews; proactive recall for the 3 treatment targets and screening.
- Brent Health Matters outreach and community-venue clinics.

How effectiveness would be evaluated

Measure	Baseline	Target (12-18 months)
3 diabetes treatment targets achieved	Establish	At/above benchmark
Retinal and foot screening uptake	Establish	Improving
Equity gap vs least-deprived cohort	Measure	Narrowing

No patient-identifiable information is used; this is an illustrative model.